

WEEK 2: INTRO TO PT & MED TERM

PHYSICAL THERAPY AS A PROFESSION & ROLES OF A PHYSICAL THERAPY

- Primary role of a PT involves direct patient care
 - Standards of Practice for PT - core document

PRIMARY, SECONDARY, AND TERTIARY CARE

Primary care

- *First level of health care*
- Given by health care provider who is responsible for handling most of the person's health needs
- Family and community members may also give care

Secondary care

- A person is referred by their primary care provider to another clinician

Tertiary care

- Services are given by specialist
- Usually in facilities that focus on specific health problems
- Usually reached by referral
 - PTs are usually in secondary (referral) or tertiary care (specialized center)
 - PT is becoming an entry point for health care
 - Direct access - practice without referral

APTA CORE VALUES & PROFESSIONAL BEHAVIORS

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1.) Social responsibility

- Promotion of mutual trust between the profession and the larger public that necessitates responding societal needs for health and wellness

2.) Professional duty

- Commitment to meeting one's obligations to provide effective PT services to clients

3.) Accountability

- Taking responsibility for own actions

4.) Altruism

- Placing the needs of patients and clients ahead of PT or PTA's self interest

5.) Compassion

- Demonstrating empathy, concern, and consideration for the needs and values of others

6.) Integrity

- Adherence to high ethical standards or principles, being truthful, ensuring fairness, following through commitments, and verbalizing to others the rationale for actions

7.) Excellence

- Striving for high standards and doing your best at all times
- Quality treatment
- Further expanding of existing knowledge

PRACTICE SETTINGS OF PHYSICAL THERAPY

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- 1.) Community-based rehabilitation
- 2.) Research and academic institutions
- 3.) Schools
- 4.) Sports facilities & fitness centers
- 5.) Home health care
- 6.) Hospitals
- 7.) Private clinics

APTA SPECIALIZATIONS IN PHYSICAL THERAPY

SCOPE NG Women

1.) Sports physical therapy

- Specializes in prevention, evaluation, and rehabilitation of sports-related injuries; works with athletes

2.) Cardiopulmonary

- Focuses on patients or clients with heart and lung conditions
 - Post-heart surgery, cardiac rehab, etc.
 - Helps improve endurance, breathing, and overall cardiovascular health

3.) Orthopedics

- Focuses on musculoskeletal conditions
 - Includes post-surgical rehab arthritis, back/neck pain

4.) Pediatrics

- Works with infants, children, and adolescents

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- Focuses on developmental delays, congenital conditions, and movement disorders

5.) Neurology

- Works with patients who have neurological disorders
 - CVA, PD, MS, SCI, TBI

6.) Geriatrics

- Focuses on older adults/elderly and age-related conditions
 - Includes fall prevention, balance training, mobility, and management of chronic diseases

7.) Women's health

- Focuses on pelvic floor dysfunction, pregnancy/postpartum care, incontinence, and pelvic pain

PROFESSIONALISM IN PT PRACTICE

- 1.) Adherence to ethics and legal standards
- 2.) Lifelong learning and continuing education
- 3.) Effective communication with patients and health teams
- 4.) Advocacy for patient rights and accessibility

PROFESSIONALISM CHALLENGES IN PT PRACTICE IN THE PHILIPPINES

- 1.) Limited equipments and resources
- 2.) Work load > Salary
- 3.) Cultural and language barriers
- 4.) Ethical dilemmas
- 5.) Patient with unrealistic expectation
- 6.) Patient with no show
- 7.) Balancing personal emotions with patients

APTA ROLE OF PHYSICAL THERAPISTS CCAPE

1.) Critical inquiry/Research

- Engaging in scholarly activities such as research, evidence-based practice, and the application of scientific knowledge to improve clinical decision-making and patient outcomes

2.) Consultation

- Providing patients and clients expert advice, recommendations, and professional judgment without necessarily providing direct patient care

3.) Administration

- Managing, organizing, and directing PT services, programs, or facilities
- Include leadership, supervision, and policy development
- Planning, communicating, delegating, managing, directing, supervising, budgeting, evaluating

4.) Patient/Care Management Model

a.) Examination

- First step: collection information about the patient
 - History: involves the past and current condition or health status
 - System review: Quick check of the body's main system to know whether referral is necessary (musculoskeletal, neuromuscular, cardio/pulmo, integ, cognitive function)
 - Tests and measures: Specific procedures to measure physical and functional status

b.) Evaluation

- PT reviews data after examination to make a clinical judgment
- Sometimes involves consulting with other professionals

c.) Diagnosis

- PT establishes a diagnosis by categorizing the finding based on the evaluation
- Follows APTA policy: PTs professional judgment and responsibility to refer patients

d.) Prognosis

- PT predicts the expected level of improvement and long it will take
- Creates a plan of care:
 - Short-term and long-term care, expected outcomes, specific interventions, discharge criteria

e.) Intervention

- Carry out the plan of care. It has three parts:
 - Coordination, communication, and documentation
 - PTs must communicate effectively (oral, written, nonverbal) with patients, families, referral sources, and other professionals

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- Documentation is essential for permanent record for future reference and required by regulation and insurance
 - Narrative (flexible, unstructured)
 - Standardized forms (efficient but less flexible)
 - SOAP notes by Reed 1969
- Nonverbal: impact comes from facial expression (55%), voice (38%), and written (7%)
- Patient/Clinical-related instruction
 - Provides education and training for patients/clients and caregivers
- Procedural interventions
 - Main therapeutic interaction
 - Includes manual techniques (high-touch) and equipment-based (high-tech)

f.) Reexamination and Termination of Services

- If goals are not met, PTs may conduct reexaminations or may be referred
- Must be documented
- Services end in two ways:
 - Discharge - when goals are achieved
 - Discontinuation - when patient chose to stop, medical or financial reasons, PTs referral

5.) Education

- Educating patients, families, students, colleagues, and the community to promote health, prevent disability, and ensure effective practice of PT